# A Plan You Make Before You Need It

A short guide to what it contains: what to do now, what to watch and when to pick it up again.

Everyone says families should plan ahead for long-term care. Hardly anyone says what the plan is supposed to have in it, so the advice tends to leave a family with a folder of statements, a few opinions from different people, and a sense that there must be something to do.

This guide describes, in general terms, what a plan made ahead of time contains, and why much of it is watching rather than doing. It explains. It doesn't tell your family what to do or when.

## It starts as a picture

A plan made ahead of time begins as a picture of where things stand, not a list of things to do. The picture has five parts: Mom's care needs, what she and the family would prefer, her financial resources, where things stand with Medicaid, and the legal planning already in place.

Medicare.gov encourages planning early for non-medical long-term care, to maintain independence and to get the care a person may need, in the setting she wants. The same page says that Medicare and most health insurance don't pay for long-term care, which is mostly help with the basic personal tasks of everyday life. [1]

Medicaid, a joint federal and state program that helps people with low income and assets pay health care bills, is different: unlike Medicare, it does pay for custodial care in nursing homes and at home. Its overall rules are based on federal requirements, but states have considerable leeway, so eligibility rules and covered services vary from state to state. That is why this guide states no limits, and why “where does Mom stand?” is a question for a professional who knows your state. [2]

- For each of the five parts, the useful note is what you know, what you're assuming, and what nobody has looked at.
- A blank under a heading is information too. It tells you what to ask about.

## Much of it is watching, not doing

When a family has time, it's tempting to start doing things. The items in a plan like this are of three kinds. Some are organizing: gathering, listing, talking, writing things down. Some are changes: moving money, retitling property, signing or changing a legal document, buying or cancelling a policy. And some are simply things to keep an eye on.

Organizing can go at a family's own pace. Changes are where a professional comes in first. Under the heading “Medicaid is complicated,” the Consumer Financial Protection Bureau's guide for people managing a relative's money says to get legal advice and be very careful about decisions that may affect a person's eligibility for Medicaid. [3]

What can be addressed now and what should simply be monitored is worked out with your Medicaid planner. What a family can do is keep the three kinds apart on paper, so that an idea for a change is written down as a question before anyone acts on it.

- A plan made ahead of time spends most of its life waiting. That isn't the same as doing nothing: it means knowing what you're keeping an eye on.

## Who is involved, and for what

No one person answers every question in a plan like this. In this roadmap, your Medicaid planner brings the family's care needs, preferences, finances, Medicaid readiness and existing legal planning into one picture, and helps the family separate what can be addressed now from what should be monitored.

Legal questions belong to an elder law attorney. The Administration for Community Living lists the kinds of work: preparing power of attorney documents, advance medical directives and wills, exploring a person's qualifications for Medicaid, and general counsel on the issues surrounding aging. [4]

A licensed financial professional and a licensed insurance professional review the questions that belong to them, and a tax professional the tax questions. The Consumer Financial Protection Bureau's guide notes that many professionals must be licensed or registered by a government agency, and suggests checking qualifications and asking questions before hiring anyone. [3]

For care itself, a geriatric care manager is one kind of professional a family may meet: usually a licensed nurse, social worker or therapist trained in caring for older adults, who evaluates care needs and recommends services. [5]

- The useful note is each open question with the kind of professional it belongs to written beside it.
- When each professional becomes necessary depends on the family. It is one of the questions to bring to your Medicaid planner.

## What would make you pick it up again

What makes a waiting plan useful is knowing in advance which changes mean it's time to open it again. The kinds of change a family might put on that list are a hospital stay, a new diagnosis, changes in memory, needing more help with daily activities, a family caregiver needing more support, a move to a care facility, and care costs rising quickly.

“Daily activities” has a standard meaning. The Administration for Community Living calls the basic personal tasks of everyday life activities of daily living: bathing, dressing, using the toilet, moving to or from a bed or chair, caring for incontinence, and eating. [6]

Which events belong on your family's list, and what happens when one occurs, is settled with your Medicaid planner, not by this guide. The habit behind it is a general one: the Family Caregiver Alliance says family meetings need to take place regularly, and at least when the caregiving situation or other situations in family members' lives change. [7]

## What this means for your family

- “Plan ahead” becomes four things you can describe: the picture, what is organizing and what is a change, who is involved, and what would make you pick it up again.
- Your part is to gather, write down, notice and ask.
- Changes to money, property, legal documents or insurance go to the professional they belong to before anything is done.
- The questions that depend on your state's rules or on Mom's own situation go to your Medicaid planner, an elder law attorney, or a licensed financial or insurance professional.

**What this guide doesn't do**

It doesn't say what your family should do now, what can wait or when to act, and it doesn't state any Medicaid rule: those vary by state and change. The general statements come from the sources below. The four parts of the plan are the Medicaid Planning Roadmap's own. Nothing here is legal, financial or tax advice.

## Ready for the next step?

You know what a plan made ahead of time contains, and where a family's part of it ends. The planning interview is the next step: it asks about your family's situation and gives you a free planning report you can bring to any conversation about paying for care.

**Get Your Free Planning Report ›**

### WHERE THIS COMES FROM


1. Medicare.gov (Centers for Medicare & Medicaid Services), Long-term care
2. Administration for Community Living (LongTermCare.gov), Medicaid
3. Consumer Financial Protection Bureau, Managing someone else’s money: Help for agents under a power of attorney
4. Administration for Community Living (LongTermCare.gov), Will I Need a Lawyer?
5. AARP, Geriatric Care Managers Advocate for Older Adults — and Their Caregivers
6. Administration for Community Living (LongTermCare.gov), What is Long-Term Care?
7. Family Caregiver Alliance, Holding a Family Meeting


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